

THE CLINICAL AI BRIEF™

Practical AI insights for clinicians across healthcare, education, and care settings

ISSUE 02 | YOUR EXPERTISE IS THE POINT

A Quick Note Before We Dive In

Welcome back. If you are joining us for the first time, we are glad you are here. In Issue 01, we talked about how AI is already embedded in clinical workflows whether we named it or not. Today, we go one layer deeper.

This issue is about something that does not get said clearly enough: your clinical expertise is not a nice-to-have when it comes to AI. It is the whole point.

The Insight: AI Needs You More Than You Need It

There is a common fear circulating in healthcare right now -- that AI will replace clinicians. It is understandable. And for most of what you do, it is not accurate.

Here is why: AI is trained on patterns in existing data. In healthcare, that data reflects systems that were built before many of your patients existed in them fully. It reflects documentation practices, population samples, and clinical assumptions that your training taught you to question.

You bring something AI genuinely cannot generate on its own: clinical reasoning built on human relationship, sensory observation, cultural context, and the kind of professional judgment that comes from years of working directly with people.

What AI does well:

- Synthesizing large amounts of structured data quickly.
- Flagging patterns across populations that a single clinician might miss.
- Reducing time spent on repetitive documentation tasks.
- Generating first-draft content for clinicians to review and refine.

What AI still cannot do:

- Read the room. Notice when a patient's affect contradicts their words.
- Apply culturally responsive, person-centered clinical judgment.
- Take ethical and legal responsibility for a clinical decision.
- Replace the therapeutic relationship that drives outcomes in your discipline.

Put It Into Practice: Three Questions Worth Asking

The next time an AI tool surfaces in your setting (in a meeting, a product demo, a new platform rollout) use these three questions to stay grounded:

1. What data was this trained on, and does it reflect my patient population?

AI tools trained primarily on certain demographic or institutional data may perform differently -- and less accurately (with diverse populations). As a clinician, you are often the person best positioned to notice when an output does not fit.

2. Who is accountable when the AI is wrong?

Spoiler: it is almost always the clinician. Understanding liability is not a legal deep-dive (it is a professional literacy issue). Know what you are signing off on.

3. Does this tool make my clinical work better, or just faster?

Speed is valuable. Quality is non-negotiable. The best AI tools in clinical settings do both. Tools that only speed things up without improving accuracy or reducing burden deserve scrutiny.

Your Reflection for This Week

Think of one clinical skill or judgment call that you make routinely, something that took years to develop. Now ask: could an AI tool replicate it accurately enough to be trusted in your setting? Sit with the answer. And notice what it tells you about what makes your work irreplaceable.

COMING UP IN ISSUE 03

We will look at AI and clinical documentation -- specifically, what ambient AI scribes are, how they work in speech-language pathology and rehabilitation settings, and what clinicians should know before their employer rolls one out.

You are part of a growing community of clinicians thinking carefully about AI in practice.

Have a question or want to suggest a topic? Reach us at: newsletter@clinicalaicollective.net

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